



POLICY

Medication: Off-Label Prescribing Including Prescribing Greater than Maximum Dose

Scope (Staff):	North Metropolitan Health Service Mental Health (NMHS MH): Clinical Staff
Scope (Area):	NMHS MH: Adult Services

1. Aim

To ensure consistent and evidence-based practice for prescribing and administering medication outside of the registered product information. For the purposes of this policy, “*medication*” refers to both psychotropic and non-psychotropic medication.

2. Background

Off-label prescribing refers to the prescription of a medicine that is used in a manner other than that specified in the Therapeutic Goods Administration (TGA) approved product information. This includes when the medicine is prescribed:

- for another indication;
- at a different dose;
- via an alternate route of administration;
- for a patient of an age or gender outside the registered use.

Off-label prescribing occurs on a spectrum. When there is high quality evidence to support use or prescribing is very slightly out of the realm of license and has a favourable risk/benefit ratio, off-label prescribing is generally considered routine or low risk. Where there is low quality evidence to support the use and/or prescribing is very far out of the realm of license and/or the risk/benefit ratio is less favourable, prescribing is generally considered as exceptional use.

In all situations, prescribers must demonstrate that the potential benefits outweigh the potential harm, that the risks have been evaluated and methods for assessing efficacy of the off-label prescribing are planned.

For patients under the age of 18 please refer to the [WA Child and Adolescent Health Services Use of Medications for other than Manufacturer’s Approved Indications Policy](#)

Failure to comply with the principles outlined in this policy may expose the patient to risk of unnecessary harm or lack of benefit, the clinician to professional liability and the health service to financial and legal ramifications.

3. Principles

3.1 Prescribing

- The TGA approved indication, dose, route, and patient factors for a medication can be found on the TGA website ([Product Information | Therapeutic Goods Administration \(TGA\)](#)) and in the MIMS product information. Prescribing must comply with the product information unless prescribing takes place in accordance with this policy.
- Prescribers must also adhere to the requirements of the [NMHS MH Formulary and Non-Formulary Prescribing Policy](#). Medicines that are listed on the [WA Statewide Medicines Formulary](#) (SMF) may in fact be 'off-label' with reference to the manufacturer's instruction.
 - An individual patient non-formulary request form must also be completed for medications not on the SMF as per the NMHS MH Formulary and Non-Formulary Prescribing Policy.
- Off-label prescribing may only occur on the authority of a consultant psychiatrist or an appropriate specialist.
- Informed consent from the patient and/or guardian must be obtained and documented.
 - If informed consent cannot be obtained from an involuntary patient, a second opinion from another consultant psychiatrist must be obtained.
 - Complex and high risk situations should involve the Head of Clinical Service and/or the Chief Pharmacist in the decision-making process.
- The rationale and plan for treatment must be documented in the patient's integrated progress notes and must include:
 - the reason for the off-label medication, and evidence supporting its use;
 - other strategies that have been tried and failed;
 - a summary of risks that have been evaluated;
 - discussion with the patient and/or guardian, including patient and/or guardian consent or a second opinion for involuntary patients unable to consent;
 - physical examination and investigations;
 - how treatment will be evaluated and monitored;
 - review time frames and the duration of treatment.
- Prescribers should consider formally publishing findings (eg. case study or series) in order to contribute to an evidence base.

3.2 Prescribing Greater Than Maximum Dose

- Prescribing greater than maximum dose is a type of off-label prescribing that refers to the prescription of a single medication that exceeds the licensed maximum dose.
- It may also refer to the prescription of two (or more) medications from the same class (e.g. antipsychotics) such that when the individual doses are expressed as a percentage of their maximum licensed dose, the sum of the percentages is greater than 100%.
 - This is considered 'off-label' prescribing for the purposes of this policy even if one or all medications assessed individually do not exceed the licensed dose.

- Of particular relevance to mental health are antidepressant and antipsychotic classes of medication.
- Guidance on antidepressant and antipsychotic maximum doses is located in [Appendix 1](#).
- A documented plan for close physical monitoring and identification of adverse effects is essential as many side effects are dose related therefore prescription of high or combined doses are likely to result in an increased side effect burden.

3.3 Consumer Information

Patients and guardians must be provided with sufficient verbal and written information regarding the treatment to enable informed decision making. Carers must be provided with information to facilitate monitoring and identification of side effects.

- It is important to counsel patients, guardians and carers on any inconsistencies in written information provided to prevent unnecessary concern or confusion.
- The [Choice and Medication](#) website may be used as a resource to provide patients with information on psychotropic medications. Many of the medication leaflets refer to common off-label indications.
- Formulary One has simplified Medication Information Leaflets (MILs) for a range of non-psychotropics medications.

3.4 Transfer of Information to Healthcare Providers

At transfers of care, receiving healthcare providers must be fully informed and appropriate documentation received that an off-label medication has been prescribed, the duration of treatment and any ongoing monitoring requirements.

4. Compliance and Evaluation

- Compliance with this policy will be assessed via the Medication Safety audit cycle by:
 - Drug Utilisation Audit of Higher than Maximum Licensed Dosing (NMHS MH pharmacy)

References

[NMHS MH Formulary and Non-Formulary Prescribing Policy](#)

[WA Statewide Medicines Formulary](#)

[CATAG: Guiding principles for the quality use of off-label medicines](#)

[WA Child and Adolescent Health Services Use of Medications for other than Manufacturer's Approved Indications Policy](#)

[MIMS Online](#)

[The Royal Australian & New Zealand College of Psychiatrists](#)

Policy Sponsor	NMHS MH Chief Pharmacist				
Policy Contact	NMHS MH Chief Pharmacist				
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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 3: Consumer and Carer Participation		☒ Std 8: Governance, leadership and management ☒ Std 10: Delivery of Care		
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management,				
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Appendix 1: Antidepressant and Antipsychotic Approved Maximum Doses

Antidepressant	Maximum Daily Dose (Manufacturer's Product Information)
Agomelatine	50 mg
Amitriptyline	300 mg
Citalopram	40 mg
Clomipramine	150 mg
Desvenlafaxine	200 mg
Dosulepin (dothiepin)	200 mg
Doxepin	300 mg
Duloxetine	120 mg
Escitalopram	20mg
Fluoxetine	80mg
Fluvoxamine	300mg
Imipramine	300mg
Mianserin	120mg
Mirtazapine	60mg
Moclobemide	600mg
Nortriptyline	100mg
Paroxetine	60mg
Reboxetine	10mg
Sertraline	200mg
Tranlycypromine	30mg
Venlafaxine	225mg
Vortioxetine	20mg

Antipsychotic (ORAL)	Maximum Daily Dose (Manufacturer's Product Information)
Amisulpride	1200mg
Aripiprazole	30mg
Asenapine	20mg
Brexpiprazole	4mg
Cariprazine	6mg
Chlorpromazine	800mg
Clozapine	900mg
Haloperidol	20mg
Lurasidone	160mg
Olanzapine	20mg
Paliperidone	12mg
Periciazine	75mg
Quetiapine	800mg
Risperidone	6mg
Ziprasidone	160mg
Zuclopenthixol	75mg

Antipsychotic Long Acting Intramuscular (Depot) Injection	Maximum Daily Dose (Manufacturer's Product Information)
Aripiprazole Monohydrate (Abilify Maintena®)	400mg / 4 weeks
Flupentixol (Flupenthixol) Decanoate (Fluanxol Depot®)	100mg / 2 weeks
Haloperidol Decanoate (Haldol®)	300mg / 4 weeks
Olanzapine Pamoate (Zyprexa Relprevv®)	300mg / 2 weeks
Paliperidone Palmitate (1 monthly) (Invega Sustenna®)	150mg / 4 weeks
Paliperidone Palmitate (3 monthly) (Invega Trinza®)	525mg / 12 weeks
Paliperidone Palmitate (6 monthly) (Invega Hafyera®)	1000mg / 6 months
Risperidone (Risperidone Consta®)	50mg / 2 weeks
Zuclopenthixol Decanoate (Clopixol® Depot)	400mg / 2 weeks

Antipsychotic Short Acting Intramuscular Injection	Maximum Daily Dose (Manufacturer's Product Information)
Droperidol	25mg per dose May be repeated at 4-6hrs
Haloperidol (Serenace®)	10mg per dose 20mg/day
Olanzapine (Zyprexa IM®)	10mg per dose 30mg/day
Ziprasidone (Zeldox®)	20mg per dose 40mg/day
Zuclopenthixol Acetate (Clopixol Acuphase®)	150mg per dose Dose not to be repeated < 24 hrs 400mg total in 2 weeks